

U.S. Healthcare Services

US Healthcare Svcs - Hospital payrolls growth continues at lower levels



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Supply-side indicators continue to support a normalized, modest utilization backdrop, with hospital capacity growth slowing further while ambulatory hiring has stabilized off recent lows. On payrolls, September continues to show slower capacity growth than in recent years — nearing traditional levels (hospital payroll growth at its lowest since mid-2022). We continue to see some potential for this to continue to slow to below the LT averages, with hospital payroll still 2.8% above the LT trend line.

While wage inflation is returning to more historic levels, temp labor continues to rise, which we see as an implication that hospitals still are focused on managing costs first as opposed to planning for step-ups in volume.

September payroll growth remained at lower historic levels. Over the past year, total payroll growth for hospital and ambulatory settings has dropped from a peak of 4.2% year-over-year in May '24 to 2.2% in Sep'26. Payrolls have now met the pre-COVID trend line for combined ambulatory and hospital payrolls ([click here](#) for our updated model).

Combined hospital and ambulatory payrolls are now at historic trend line. Hospital payrolls are now ~2.8% above the historic trend line, while ambulatory payrolls are ~2.3% below trend line. Hospital payrolls were nearly 5% below the pre-COVID trend in Jan '22, and have since grown at 2.5-3x the historic rate to surpass this trend line by ~156k employees (~2.8%). The key question for hospitals will be if this is a stable level of capacity, or if capacity growth may slow further to reflect lower insurance coverage.

Implications - We continue to see this as neutral to modestly positive for MCOs, as utilization which is stable and closer to historic levels would be favorable to conservative pricing assumptions but likely consistent with market expectations.

The capacity trend continues to be slightly negative for hospitals, with concerns over future volumes persist. Coupled with changes in insurance coverage levels (which will also continue in 2027), the capacity changes could work together to drive a continued slower volume environment 2026 and into 2027 for hospitals.

BERNSTEIN TICKER TABLE

Ticker	Rating	5 Oct 2026		Price Target	TTM Rel. Perf.	Adjusted EPS				Adjusted P/E (x)		
		Cur	Closing Price			Cur	2025A	2026E	2027E	2025A	2026E	2027E
AGL (agilon health)	O	USD	84.28	125.00	182.6%	USD	(24.46)	(5.34)	(2.38)	(3.4)	(15.8)	(35.4)
CNC (Centene)	O	USD	64.30	79.00	50.4%	USD	2.08	5.05	5.43	30.8	12.7	11.8
CI (Cigna)	O	USD	271.29	387.00	(28.5)%	USD	29.84	30.48	33.56	9.1	8.9	8.1
CVS (CVS)	O	USD	87.01	112.00	(3.5)%	USD	6.75	8.03	8.45	12.9	10.8	10.3
ELV (Elevance)	O	USD	394.51	488.00	(3.2)%	USD	30.30	27.09	30.60	13.0	14.6	12.9
HCA (HCA)	M	USD	434.92	426.00	(14.5)%	USD	28.21	29.92	30.68	15.4	14.5	14.2
HUM (Humana)	O	USD	404.07	425.00	26.7%	USD	16.60	9.45	15.40	24.3	42.7	26.2
MOH (Molina)	O	USD	194.05	266.00	(19.7)%	USD	11.03	5.28	10.68	17.6	36.7	18.2
UNH (UNH)	O	USD	378.58	512.00	(10.7)%	USD	16.35	20.11	22.11	23.2	18.8	17.1
SPX			7,773.95									

O - Outperform, M - Market-Perform, U - Underperform, NR - Not Rated, CS - Coverage Suspended

Source: Bloomberg, Bernstein estimates and analysis.

INVESTMENT IMPLICATIONS

We rate AGL Outperform with a target price of \$125, CNC Outperform with a target price of \$79, CI Outperform with a target price of \$387, CVS Outperform with a target price of \$112, ELV Outperform with a target price of \$488, HCA Market-Perform with a target price of \$426, HUM Outperform with a target price of \$425, MOH Outperform with a target price of \$266, and UNH Outperform with a target price of \$512.



DETAILS

Supply side suggests stable and modestly slower utilization - We are seeing 1) care delivery capacity growth back near historical levels and modestly decelerating; 2) slowing wage inflation; and modestly higher temporary labor. We expect this indicates care delivery utilization is normalizing and continuing to modestly decelerate, while hospitals and physician practices are expecting slow volume growth and are managing down labor costs. The modestly higher temporary labor suggests to us that providers are expecting future volumes to be lower, thus justifying higher temporary labor rather than higher full-time employment costs.

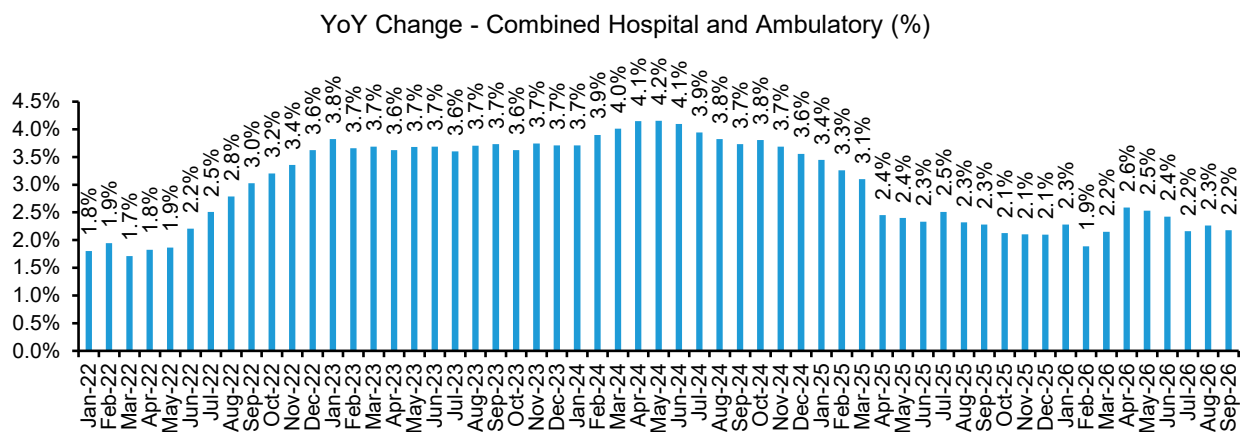
Total payroll growth for hospital and ambulatory settings has dropped from a peak of 4.2% year-over-year in May '24 to 2.2% in Sept '26 (click [here](#) for our updated model). Hospital payroll growth has been hovering below 3%, down from a peak of 4.3% in Mar '24. At 1.9%, Sept '26 year-over-year growth was the lowest since that peak. Ambulatory payroll growth was 2.4% in Sept '26 vs. ~3.5% to 4+% over the past couple of years. We track both data sets, as the combined environment best describes overall care provider capacity. Overall, trends across payrolls have remained consistent over the past several months.

Combined hospital and ambulatory payrolls are now at historic trend line. Hospital payrolls are now ~2.8% above the historic trend line, while ambulatory payrolls are ~2.3% below trend line. Hospital payrolls were nearly 5% below the pre-COVID trend in Jan '22, and have since grown at 2.5-3x the historic rate to surpass this trend line by ~156k employees (~2.8%).

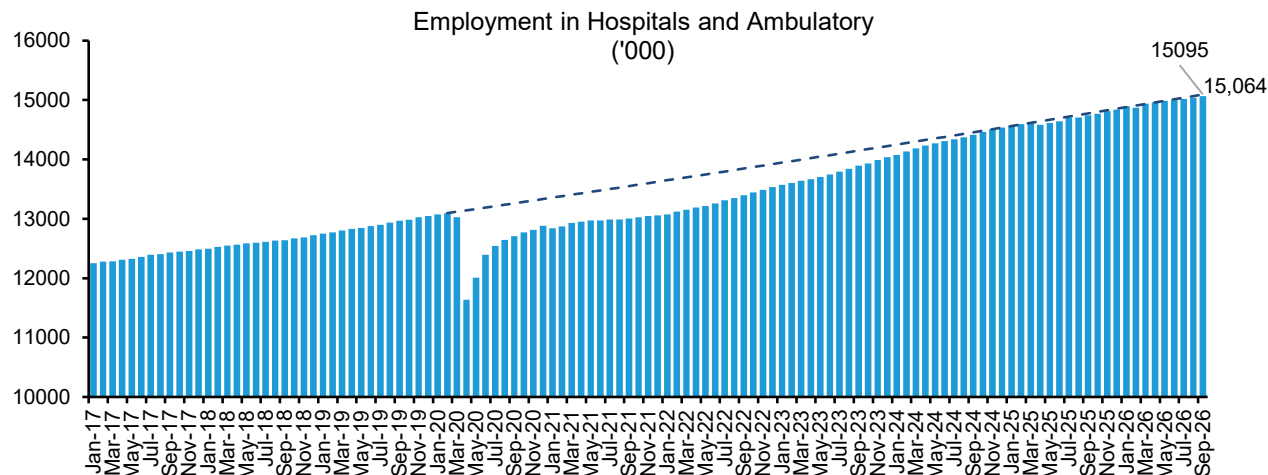
We believe the 2023/2024 acceleration in utilization was driven by labor supply growth, and we expect the normalization of payroll growth to lead to decelerating utilization over the near term. We believe that demand for healthcare services had largely returned by early 2022, but it was limited by labor supply. At that time, hospital payrolls were at ~95% of the pre-COVID trend, suggesting an under-supply of ~5%. Hospitals were successful in drawing healthcare professionals back to the workforce by increasing wages, which grew by an average rate of 6.7% in 2021-2022, ~2.5x the rate seen in pre-pandemic years. This led to sharp acceleration of payroll growth, with monthly payrolls increasing at an average rate of 3.0% year-over-year since Jan '23, over twice their historic average. We believe this led to the sharp increase in utilization over this time period.

Payroll growth continues to be relatively flat. Over the past year, total payroll growth for hospital and ambulatory settings has dropped from a peak of 4.2% year-over-year in May '24 to 2.2% in Sept '26. Payrolls have now met the pre-COVID trend line for combined ambulatory and hospital payrolls.

EXHIBIT 1: Hospital and Ambulatory payrolls - YoY Changes %



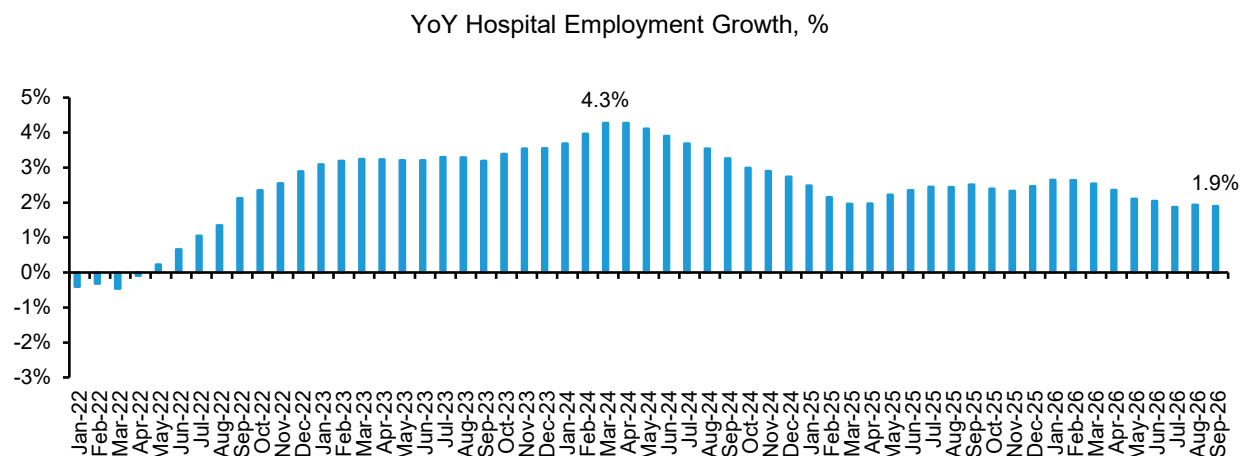
Source: BLS, Bernstein analysis

EXHIBIT 2: **Employment in Hospitals and Ambulatory combined**

Source: BLS, Bernstein analysis

HOSPITAL EMPLOYMENT

Hospital employment growth has been declining in 2026 and Septembers sees the trend continue. Hospital payroll growth has been slowly declining over the past several months, maintaining a rate below 3%. Sept '26 added 108k hospital jobs vs. Aug '26, in-line with the decelerating levels we saw last year. On a rolling 12-month basis, hospital employment increased 1.9% YoY in Sept '26, continuing the declining trend that began in May '24. Monthly payroll additions were around 12k, which is on par for 2026 and about 35% below average levels from 2025 and 2024.

EXHIBIT 3: **Hospital employment - YoY Change, %**

Source: BLS, Bernstein analysis

As opposed to the 2Q26, which saw the lowest level of hospital employment growth since 1Q22, 3Q26 hospital employment growth saw a rebound back to more consistent levels (growing 0.46% QoQ).



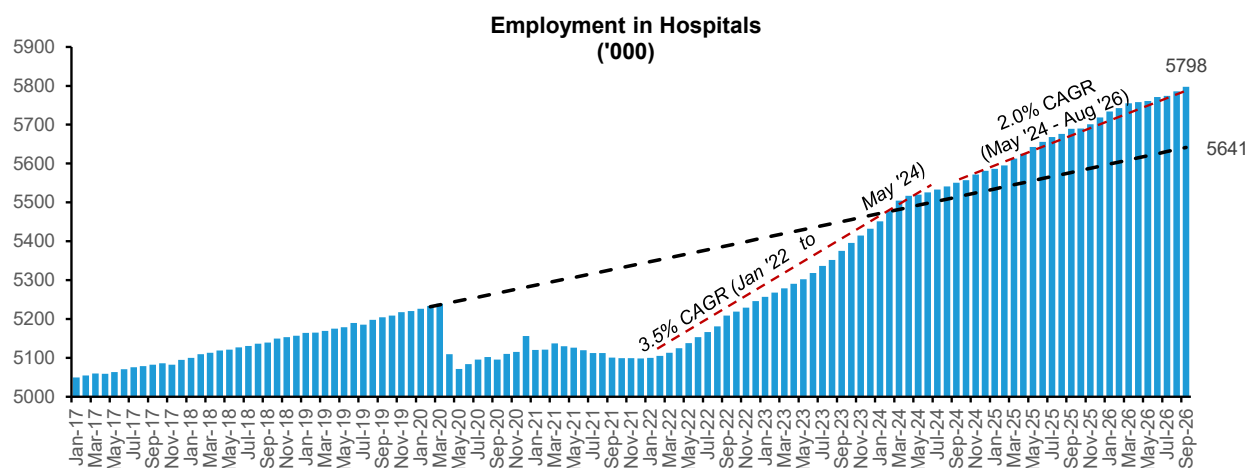
EXHIBIT 4: Hospital employment - YoY Change, %

A closer look at past 3 years shows the extent of slowing



Source: BLS, Bernstein analysis

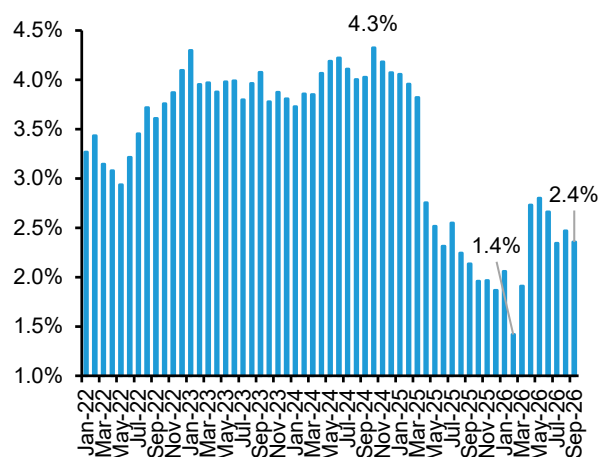
Hospital payrolls surpassed the pre-COVID trend line in February '24 and are currently ~2.8% above the trend line. We continue to believe the high growth rate in volumes has been aided by this rapid expansion in capacity starting in early 2022. We would expect that this growth in capacity will slow back toward LT historic trends of 1.5% over time, which suggests risks to the downside for utilization as we move through 2026.

EXHIBIT 5: Hospital employment

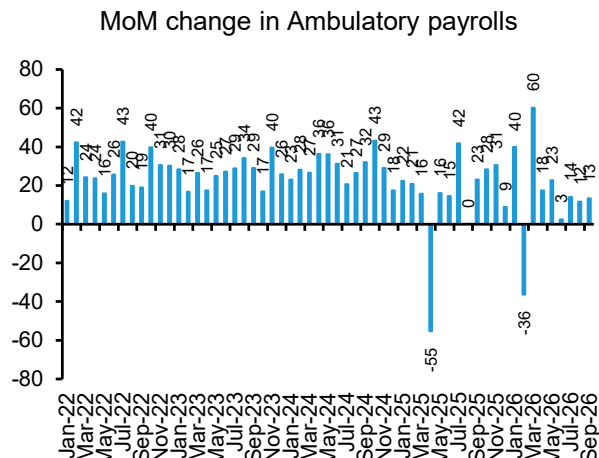
Source: BLS, Bernstein analysis

AMBULATORY EMPLOYMENT

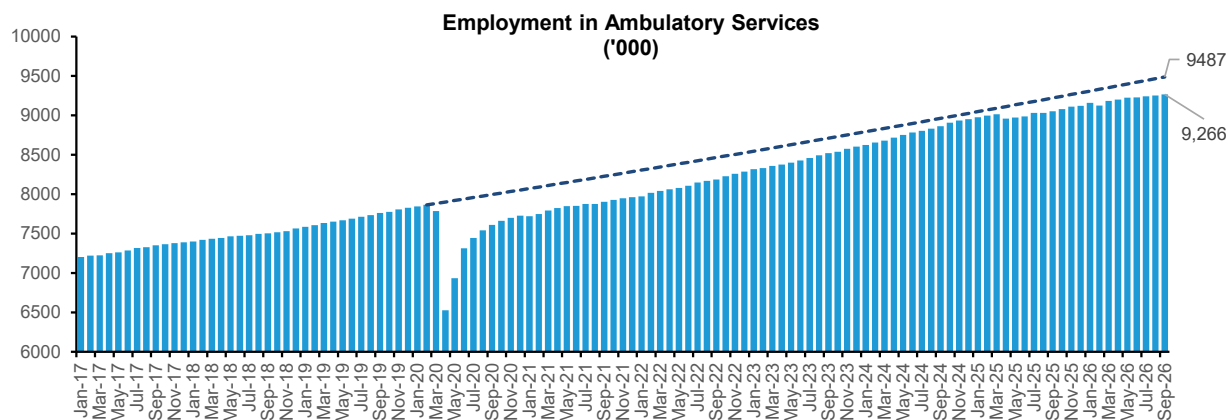
We also monitor ambulatory employment measures, which have been moving broadly in tandem with hospital payrolls. Ambulatory payrolls involve physician practices, surgery centers, and other outpatient-type businesses. Ambulatory payroll growth also stepped up following a drop and then restraint during COVID. Ambulatory payrolls have been growing above the pre-COVID trend (generally around 1.5-2x) vs. hospital payrolls which have been growing at ~3x the pre-COVID trend. Ambulatory payroll growth has been higher in recent months compared to the beginning of the year, after dropping below 2% in Dec '25 from peak levels of over 4%. Year-on-year growth for April to September 2026 has been around ~2.6%.

EXHIBIT 6: **Ambulatory employment - YoY Change, %**

Source: BLS, Bernstein analysis

EXHIBIT 7: **Ambulatory Employment MoM Change, (k)**

Source: BLS, Bernstein analysis

EXHIBIT 8: **Ambulatory Employment - ~2% below trendline**

Source: BLS, Bernstein analysis

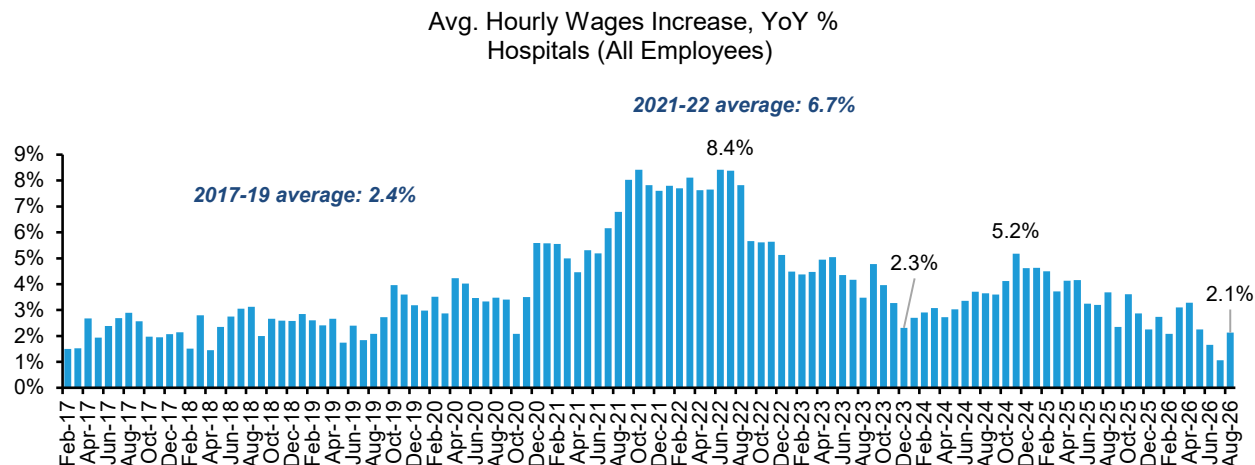
WAGE GROWTH

Hospital wages have cooled down since early 2025 and appear close to pre-COVID growth rates. We believe that higher wages were responsible for coaxing nurses and other workers back to the hospital labor force. On average, hospital sector wages grew ~2.4% year-over-year prior to the pandemic vs. an average of 4.1% in 2023. We note that hospital sector wages increased by an average of 6.7% in 2021/2022, largely due to elevated use of expensive temporary labor and permanent wage inflation.

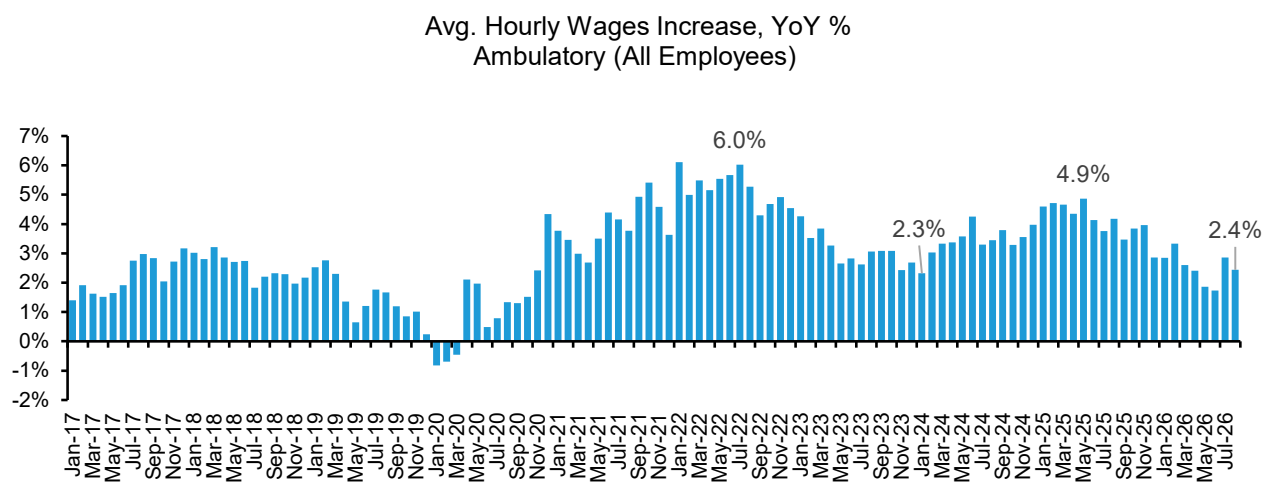
Hospital wage growth was 2.1% in Aug '26, which is materially higher than July (1.1%). That said, these levels lie around the historic pre-COVID levels of ~2.4%. While it may still be early to call a full reversal of wage inflation declines, we see this likely signifying a reversion to historic wage inflation levels.

For context, wage growth has been declining since the high of 5.2% in Nov '24. We believe that elevated levels of wage growth over the past several years (7.1% in 2022) attracted workers and helped drive higher employment growth. We also note that ambulatory wage inflation has declined meaningfully after reaching ~5.0% in May '25. These wage inflation declines were likely a product of providers managing compensation costs as volumes slowed. We see the recent modest uptick in temporary labor as suggesting providers are seeking more flexibility rather than hiring full-time employees.



EXHIBIT 9: **Hourly Wages for Hospital Employees**

Source: BLS, Bernstein analysis

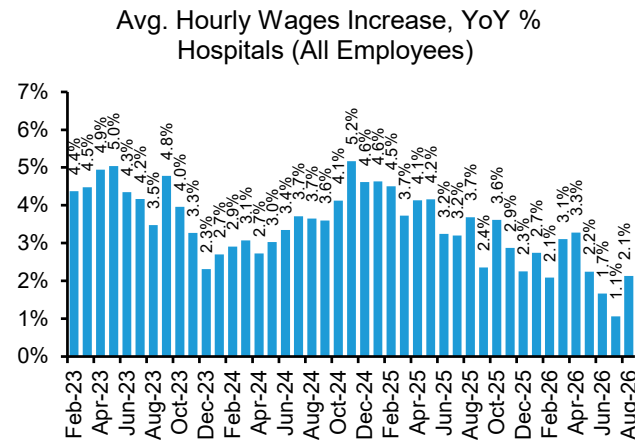
EXHIBIT 10: **Hourly Wages for Ambulatory Employees**

Source: BLS, Bernstein analysis

Last month, we noted that wage inflation was at lower levels, hitting marks not seen since 2019/2020, consistent with a slowing volume environment. In August, hospital wage inflation has returned to levels seen in May 2026. While providers are still dealing with a slower volume environment and comp expense management, we see this as a reversion to historical levels. On the other hand, ambulatory wage inflation has decreased, after having jumped up in July.

EXHIBIT 11: **Hospital wage inflation - YoY changes**

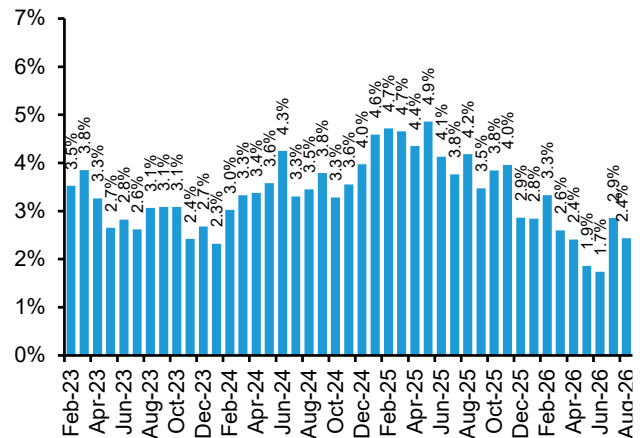
The most recent three years shows new lows in wage inflation



Source: BLS, Bernstein analysis and estimates

EXHIBIT 12: **Hourly Wages for Ambulatory Employees**

Avg, Hourly Wages Increase, YoY %
Ambulatory (All Employees)



Source: BLS, Bernstein analysis

UNEMPLOYMENT AND TRAVEL NURSE JOBS

Healthcare professionals remain one of the most in-demand segments of the U.S. labor market

The hospital labor market remains tight. Elevated travel nurse hiring helped respond to nursing shortages during COVID and exacerbated the shortage of full-time nurses by luring away full-time workers to higher paying temp roles. This trend began to reverse in late 2022, and currently, travel nurse jobs are roughly at pre-COVID levels.

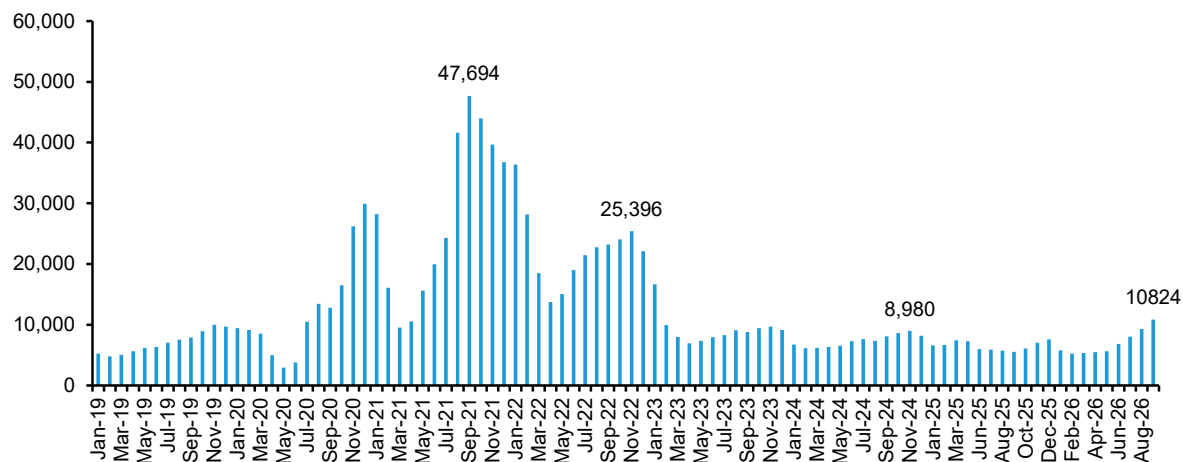
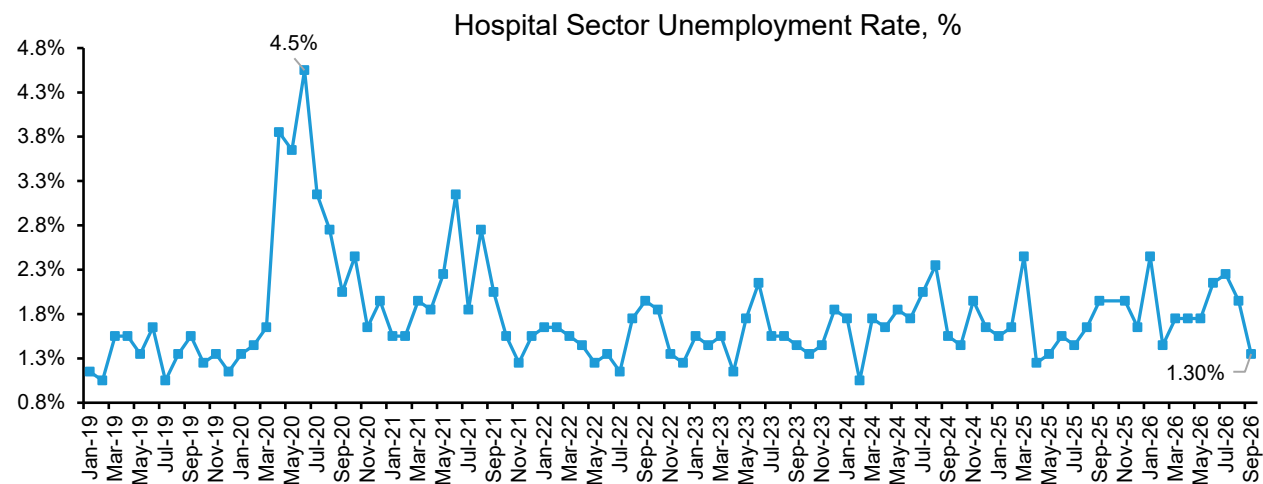
EXHIBIT 13: **National Travel Nursing Job Volume**

EXHIBIT 14: **Hospital unemployment rate**

Source: BLS, Bernstein analysis

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S. HEALTHCARE SERVICES

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